

Healthcare Marketing & HIPAA

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Medical Marketing has evolved dramatically over the years, shifting from physician-focused advertising to highly targeted digital outreach campaigns. While modern marketing offers healthcare organizations powerful opportunities to reach patients, the use of tracking pixels raises significant concerns – potentially violating privacy laws and eroding patient trust, as seen in recent hospital cases. Key legal frameworks, including the Health Insurance Portability and Accountability Act of 1996 (HIPAA), The Privacy Act of 1974, and the precedent set in *Goldfarb V. Virginia State Bar* of 1975, intersect with federal oversight by the Federal Trade Commission (FTC) to shape the boundaries of healthcare marketing practices (Gale 2019). In today’s technologically advanced arena, the line between privacy and marketing thins daily. This paper examines the historical and legal context of privacy and healthcare marketing, with a focus on hospitals’ use of website visit tracking technologies – and how these tools may contribute to HIPAA violations as we digitize more of our world.

HIPAA restricts how Protected Health Information (PHI) – including medical records and demographics – can be collected, used, and shared (Herold & Beaver, 2014). The Privacy Act governs how federal agencies manage personal data (HHS, 2025), particularly with third-party sharing. Historically, the “paternalistic nature of the physician-patient relationship” meant pharmaceutical companies marketed to doctors, who then recommended drugs directly to their patients as needed (Weinmeyer 2013). It was believed that this system drove costs up and limited patient autonomy. The Supreme Court’s decision in *Goldfarb V. Virginia State Bar* dismantled many advertising restrictions, including a section nearly identical to the Hippocratic Oath, which protected the ‘best interests of the patient’ (Tomyecz 2006). To some, this opened the marketing floodgates.

Fast forward to the present day, and consider online medical marketing systems. One of the challenges to patient privacy in online medical marketing includes something called pixels. Tracking pixels, also known as “web beacons” or “pixel tags” are tiny snippets of code

embedded into webpages that collect and transmit user data to third-party servers (HHS OCR, 2024). While invisible to the user, these pixels can capture information such as IP address, browser type, webpages visited, button clicks, and form submissions. When placed on healthcare related websites, this technology raises significant privacy concerns because it may transmit data that qualifies as PHI under HIPAA, including indicators of medical conditions and/or treatments (Friedman et al. 2024).

One of many problems with these pixels is that the user has no idea they are being tracked. This is a HIPAA concern because an individual has a right to privacy, and especially to protect their most sensitive information from being shared. Particularly when that information is being sold to companies that will use their weaknesses and medical needs against them under the guise of helping. Targeted ads generated through pixels exploit individuals' personal vulnerabilities, enticing them to purchase products that may not even help them, but simply have good advertising and insider information to draw interest (Downing, Perakslis 2022).

Under HIPAA, PHI may only be shared with entities that have a signed Business Associate Agreement (BAA) or with patient authorization. Major pixel providers such as Meta and Google typically do not have BAAs with hospitals, meaning any transmission of PHI to these companies risks noncompliance (HHS OCR, 2024). Friedman et al. (2023) found that 98.6% of U.S. nonfederal acute care hospital websites use third-party tracking tools, often transferring data to technology companies, social media platforms, advertising firms, and data brokers. Another analysis by Lokker in 2021 reported that one-third of healthcare websites still used the Meta Pixel, despite regulatory warnings and ongoing lawsuits (HIPAA Journal, 2024). A study by Niforatos et al. (2024) found that the more specialized and highly ranked the hospital, the more advertisement trackers were present on its website. This translates to a higher number of patients requiring more complex treatments, medications, and services that can be marketed.

These practices have led to substantial legal and financial consequences. For example, Novant Health agreed to a \$6.6 million settlement after disclosing PHI to third parties via tracking pixels (HIPAA Journal, 2024). Mass General Brigham and Dana-Farber Cancer Institute paid \$18 million in 2021 to settle similar claims (Friedman et al., 2023). State-level enforcement has also occurred; New York–Presbyterian Hospital settled with the New York Attorney General for \$300,000 over a pixel-related HIPAA violation (HIPAA Journal, 2024).

It is deeply concerning to consider that many hospitals – though originally unaware their pixels were causing PHI breaches – continued to use them without notifying their patients. This is the crux of the issue. While the original laws and regulations surrounding HIPAA, patient privacy, and marketing are becoming more obsolete with the rise of technology, the hospitals in these cases should have acted immediately and prioritized transparency the moment they became aware of the data breaches. It is unnerving to think that we live in a world where it is preferred to pay millions of dollars in fines rather than protect the personal security of thousands of people – while tech companies profit from something as simple as a patient’s web browsing. “Digital medicine technologies offer convenience and can improve outcomes for chronically ill patients navigating their health journeys” and should not lead to an unsafe environment where PHI is unknowingly sold (Downing, Perakslis 2022). The medical community should rethink its current focus on marketing, and instead prioritize patient care and personal safety.

The combination of legal risk, damage to reputation, and the potential for PHI misuse highlights why hospitals need to closely audit their websites and follow through with limiting third-party tracking. Without such measures, digital marketing tools like tracking pixels may dissolve patient trust and expose healthcare organizations to substantial liability.

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